Disease Outbreak News

Ebola disease caused by Bundibugyo virus - Democratic Republic of the Congo

21 May 2026

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Situation at a glance

On 15 May 2026, the Ministry of Public Health, Hygiene and Social Welfare, Democratic Republic of the Congo (DRC), and the Ministry of Health of Uganda declared an outbreak of Ebola Disease following the confirmation of Bundibugyo virus disease (BVD) in both countries. On 16 May 2026, the World Health Organization (WHO) Director-General determined that the Ebola disease caused by Bundibugyo virus in DRC and Uganda constitutes a public health emergency of international concern (PHEIC), as defined in the provisions of IHR. On 19 May 2026, the Director-General of WHO convened the first meeting of the IHR Emergency Committee, and temporary recommendations were issued to State Parties. As of 21 May, 746 suspected cases and 176 deaths among suspected cases were reported in DRC. So far 85 confirmed cases, including two in Uganda, and ten deaths, with one in Uganda, among confirmed cases were reported across both countries. In DRC, transmission is concentrated in Ituri, North Kivu and South Kivu provinces, with challenges in contact follow-up, insecure conditions, and inadequate isolation and referral systems complicating response efforts. Uganda has reported two imported cases with no confirmed local transmission. An American national who was working in DRC has also been confirmed positive and transferred to Germany for care. National authorities, in collaboration with WHO and partners, are implementing response measures including deployment of rapid response teams, delivery of medical supplies, strengthened surveillance, laboratory confirmation, infection prevention and control assessments, the set-up of safe and optimized treatment centers, and community engagement.

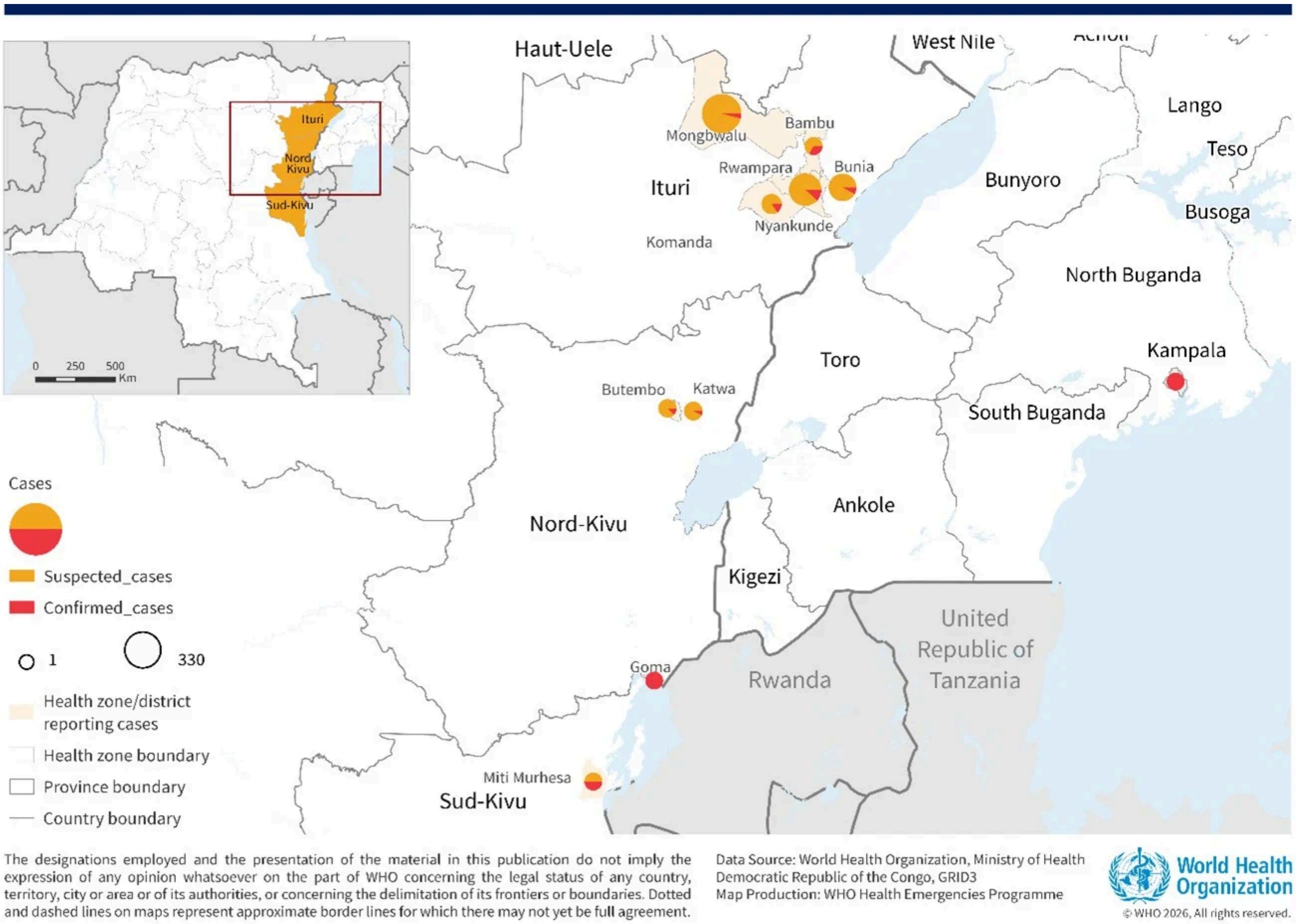
Description of the situation

On 15 May 2026, the Ministry of Public Health, Hygiene and Social Welfare of Democratic Republic of the Congo (DRC) officially declared the 17th Ebola disease outbreak following the laboratory confirmation of Bundibugyo virus disease (BVD) in eight samples. Concurrently, on 15 May 2026, the Ministry of Health of Uganda confirmed an outbreak of BVD following the identification of an imported case from DRC.

On 16 May 2026, the WHO Director-General, after having consulted the States Parties where the event is known to be currently occurring, determined that the Ebola disease caused by Bundibugyo virus in DRC and Uganda constitutes a public health emergency of international concern (PHEIC), as defined in the provisions of International Health Regulation (IHR)

Since the last Disease Outbreak News was published on 16 May 2026, the number of suspected and confirmed cases has increased rapidly in DRC, with geographical expansion into North Kivu and South Kivu. In total, 746 suspected cases, including 176 deaths among suspected cases have been reported from DRC as of 21 May 2026; and 85 confirmed cases (two in Uganda), including ten deaths (one in Uganda) (CFR 12%) have been reported from both countries.

Figure 1. Distribution of suspected and confirmed cases of Bundibugyo virus disease in Democratic Republic of Congo and Uganda, as of 21 May 2026



Democratic Republic of the Congo

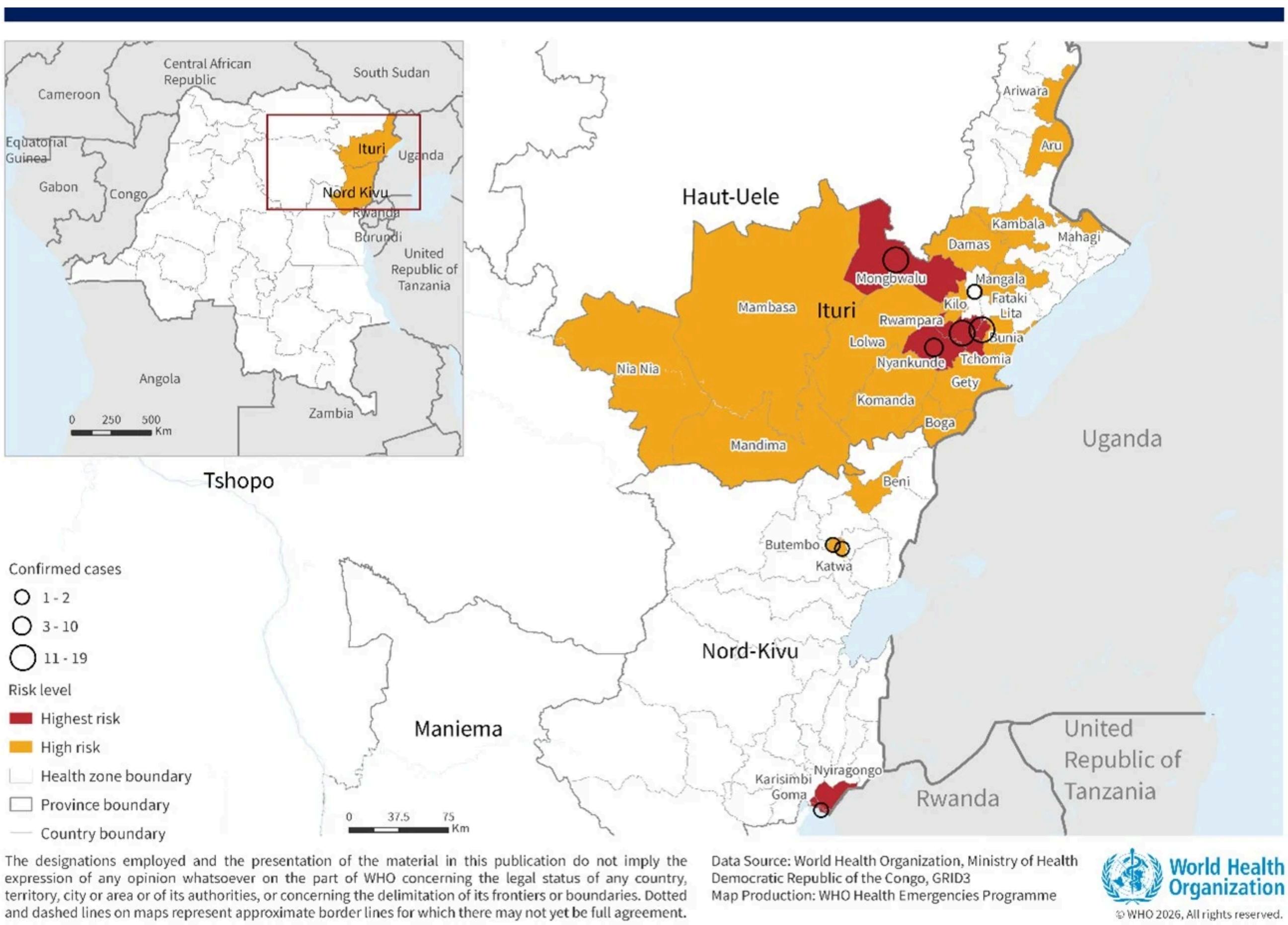
As of 21 May 2026, a total of 83 confirmed cases including nine deaths (CFR 11%); and 746 suspected cases including 176 deaths have been reported from 15 health zones (HZ) in Ituri, North Kivu and South Kivu Provinces, DRC. Four health worker deaths have been reported to date. Epidemiological and laboratory investigations are ongoing to reclassify all suspected cases and deaths reported in DRC.

The most affected HZ are Mongbwalu, Rwampara and Bunia, which all account for 96% of suspected cases and 79% of confirmed cases.

As of 21 May, 1603 contacts have been listed in Ituri province and one contact became a suspected case. However, follow-up remains weak due to insecurity and movement restrictions. The follow-up rate as of 21 May is 21%. On 21 May, 84 new alerts were reported, and 77 alerts were investigated, all of which were validated.

An American national, who was working in DRC as a surgeon, has also been identified as a confirmed case. Exposure is thought to have occurred during a medical procedure on 11 May. Onset of symptoms was reported on 16 May and laboratory confirmation was received on 20 May. The case is currently at a High-Level isolation unit in Berlin, Germany undergoing treatment.

Figure 2 Risk mapping of Health Zones in DRC as of 21 May 2026



Response efforts continue to face a number of challenges, including:

- **absence of standardized isolation and treatment facilities and weak screening and referral pathways;**
- **inconsistent implementation of safe and dignified burial measures further underscores the significant risk of healthcare-associated transmission;**
- **cross-border transmission risks remain elevated due to insecurity, humanitarian crises, high population mobility, urban/semi-urban transmission hotspots, and porous borders, requiring intensified surveillance and information sharing;**
- **deeply challenging situation for affected communities, with growing concerns over access to free and supportive healthcare services, the ability to ensure respectful and dignified burials, and the spread of misinformation and rumour;**

- **ongoing conflict in Ituri province restricting the movement of surveillance teams, the deployment of Rapid Response Teams, and the transporting of laboratory samples.**

It is currently thought that the event originated in the Mongbwalu HZ, DRC, a high-traffic mining area, with cases subsequently migrating to Rwampara and Bunia to seek medical care. Ituri province borders South Sudan and Uganda with Bunia HZ being less than 500km from Uganda. A full epidemiological investigation and trace back exercise is ongoing.

Ituri's role as a commercial and migratory hub and proximity to Uganda and South Sudan increases the risk of regional exportation and cross-border transmission.

Uganda

As of 20 May 2026, a total of two confirmed cases including one death have been reported in Kampala, Uganda. Both cases were imported from the DRC. The first case was admitted to a private hospital on 11 May and died on 14 May. The transfer of the body to DRC was completed the same day. The second case was confirmed on 16 May in Kampala, in an individual returning from DRC with no apparent links to the first case. The case is currently admitted in Uganda at the Mulago Isolation Treatment Unit. At the time of reporting, no local transmission has been identified in Uganda.

As of 18 May, a total of 127 contacts, linked to both confirmed imported cases, have been identified and under follow-up. These include close household contacts and hospital contacts where the cases were hospitalized.

Exposure risks are associated with healthcare settings and cross-border movements. Eighteen alerts were reported on 18 May and investigated. Four active cross-border exposure clusters identified in Ntoroko District are under investigation.

Epidemiology

Bundibugyo virus disease (BVD) is a severe and often fatal form of Ebola disease caused by the Bundibugyo virus, one of the *Orthoebolavirus* species. It is a zoonotic disease, with fruit bats suspected to be the natural reservoir. Human infection occurs through close contact with the blood or secretions of infected wildlife, such as bats or non-human primates, and subsequently spreads from person to person through direct contact with the blood, secretions, organs, or other bodily fluids of infected individuals or contaminated surfaces or items. Transmission is particularly amplified in health-care settings when infection prevention and control (IPC) measures are inadequate, and during unsafe burial practices involving direct contact with the deceased.

The incubation period for BVD ranges from 2 to 21 days, and individuals are usually not infectious until symptom onset. Early symptoms are non-specific, including fever, fatigue, muscle pain, headache, and sore throat, which complicates clinical diagnosis and can delay detection. These progress to gastrointestinal symptoms, organ dysfunction, and in some cases haemorrhagic manifestations. Case fatality rates in the past two BVD outbreaks, reported in Uganda and in DRC in 2007 and 2012, have ranged from approximately 30% to 50%.

Differentiating BVD from other endemic febrile illnesses such as malaria is challenging without laboratory confirmation using PCR or antigen/antibody-based assays. Control relies on rapid case identification, isolation and care, contact tracing, safe burials, and strong community engagement, as no approved vaccines or specific treatments currently exist for BVD.

Public health response

Health authorities in **DRC**, in collaboration with WHO and partners are implementing public health measures, including but not limited to the following:

Coordination

- **The Incident Management System has been activated to coordinate response to the outbreak, with technical support from WHO and health partners**
- **Subnational coordination structures are being activated at the provincial and health zones level to coordinate operational activities. Daily provincial coordination meetings involving all response pillars and operational partners are ongoing.**
- **Rapid response teams from MoH and WHO have been deployed to Bunia, Mongbwalu, and Rwampara HZ.**

Surveillance

- **Surveillance for suspected and probable cases is ongoing (including at relevant Points of Entry and borders).**
- **Alert management and case investigations are being scaled up. Investigation teams have been deployed to Bunia and Rwampara, with alerts under investigation in Ituri, North Kivu, South Kivu, and Tshopo provinces.**

- Contact tracing has been initiated with 541 contacts identified, although major operational challenges persist due to insecurity.
- Data managers have been trained on the DHIS2 tracker, and a surveillance and digital health coordination meeting is being implemented to improve harmonization across digital platforms.
- The International Organization for Migration (IOM) is supporting points-of-entry (PoEs) surveillance; however, informal crossings and weak alert management at PoEs remain significant gaps.

Case Management

- WHO and partners are supporting the ongoing establishment and operationalization of isolation and treatment facilities in affected areas
- WHO and partners are working to maintain access to essential health and other services.

Laboratory

- Laboratory surge capacity is being scaled-up. The Institut National de la Recherche Biomédicale (INRB) teams are deploying to Bunia to establish and scale-up testing. A decentralization strategy is being developed to add additional field laboratories to Mongbwalu and Mahagi (Ituri – Uganda border). Goma laboratory is activated and provide testing capacities for North Kivu.
- PCR kits have been sourced, while WHO Regional Emergency Hub in Dakar is deploying reagents, Piccolo machines, and cold-chain modules to strengthen field laboratory operations.
- Genomic and epidemiological analyses are underway, and sequences have been uploaded through a joint publication (by DRC and Uganda) on virological.org.

Risk Communication and Community Engagement (RCCE)

- Community mobilization has started in Mongbwalu, while social listening activities and deployment of UNICEF digital platforms (U-Report and I-Hear-You) are underway to improve community feedback and information sharing.
- WHO is supporting engagement interventions with community and religious leaders
- WHO shared a multi-country infodemic management report providing an initial analysis of community perceptions, including key questions, concerns, rumours, misinformation, and disinformation, to guide targeted risk communication and community engagement interventions.

WHO and partners have developed a shared RCCE message repository to harmonize risk communication content

Infection Prevention and Control (IPC)

- Coordination mechanism for IPC response is being established under the leadership of the Division of Provincial Health and the Public Health Emergency Operations Centre.
- Local human resources are being scaled-up to enable required key intervention.
- More than 150 health workers have been trained on basic IPC and Ebola-specific measures, with an ongoing cascade training plan targeting an additional 500 health workers.

- Operational teams are being established and briefed for decontamination, safe and dignified burials and health facility assessments.
- IPC supplies including PPE are being donated to priority health facilities.

Operational support and Logistics

- Over 17 tons of emergency supplies were shipped to DRC, including personal protective equipment (PPE), Viral Haemorrhagic Fever supplies, tents, body bags, infection prevention and control materials, stretchers, medicines and other case management supplies.
- Deployment of EpiShuttle patient isolation transport systems, vehicles, telecommunications equipment, laboratory consumables, portable point-of-care diagnostic machines, reagents, cold-chain modules, and Ebola polymerase chain reaction (PCR) testing kits to strengthen clinical transport, laboratory diagnostics, and field response operations are ongoing.
- Coordination is underway to mobilize one helicopter, three ambulances, and two armored vehicles to support cargo and personnel movement. Human resource deployment structures are being finalized and United Nations Humanitarian Air Service (UNHAS) is supporting staff movement to Bunia.
- Global Logistics Cluster partners briefed on situation and work is under way for planning WFP/Logistics Cluster support for common partner services.
- Efforts are ongoing with partners to provide subsidized air cargo into the region, and into Bunia
- A four-week forecast of critical PPE requirements across case management, infection, prevention and control, and burial operations has been finalized to support sustained response activities.
- A high priority items list has been finalized to facilitate collective monitoring. Item-needs calculator being finalized for sharing.

Border Health, Travel and Mass Gatherings

- WHO travel and border health guidance has been disseminated across countries and transport sectors, emphasizing that suspected, probable and confirmed cases and their contacts should avoid travel unless medically evacuated, and advising against travel or trade restrictions and border closures.
- Affected and neighbouring countries are strengthening their preparedness to detect, investigate, refer, isolate and care for any suspected cases, including activation of health emergency plans, enhanced screening at airports, seaports, land crossings and major internal transit routes.

Health authorities in **Uganda**, in collaboration with WHO and partners, are implementing public health measures, including but not limited to the following:

Coordination

- The Incident Management System has been activated to coordinate response to the outbreak, with technical support from WHO and health partners
- The National Public Health Emergency Operations Centre and regional Emergency Operations Centres (EOCs) were activated in Fort Portal, Arua, Yumbe, Kampala Capital City Authority, Kabale, and Hoima, with the national response plan and rapid risk assessment finalized.

Surveillance and Laboratory

- Field teams are utilizing Go.Data for contact tracing, benefiting from experience in implementing the tool during previous mpox, cholera and Sudan virus disease outbreaks.
- Screening is being strengthened at official and informal border crossings, major transit routes, and pilgrimage corridors.

Case Management

- Isolation facilities in high-risk districts have been activated and the Uganda National Emergency Medical Team deployed to support clinical management.

Laboratory

- Sequencing and sample transport systems are being strengthened
- A mobile laboratory is being deployed to Kasese near the DRC border, with a virtual diagnostics coordination meeting supporting cross-country laboratory operations.

Risk Communication and Community Engagement (RCCE)

- Risk communication systems have been activated with community messaging and public awareness campaigns ongoing through District Health Officer networks, with health workers receiving guidance on standard precautions and public health messaging.

Infection Prevention and Control (IPC)

- Advising health workers to remain vigilant and adhere strictly to infection prevention measures.

WHO risk assessment

On 16 May 2026, WHO Director-General, after having consulted the States Parties where the event is known to be currently occurring, determined that the Ebola disease caused by Bundibugyo virus in the Democratic Republic of the Congo and Uganda constitutes a public health emergency of international concern (PHEIC), as per the provisions of the IHR.

This is the 17th Ebola disease outbreak in the DRC since 1976. The last Ebola disease outbreak in the country was an outbreak and Ebola virus disease which was declared on 4 September 2025 with total of 64 cases (53 confirmed, 11 probable), including 45 deaths (CFR 70.3%), reported from six health areas in Bulape Health Zone, Kasai Province. The end of outbreak was declared on 1 December 2025. The last BVD outbreak was reported on 17 August 2012 by the DRC Ministry of Health in Province Orientale. A total of 59 cases, 38 confirmed and 21 probable cases, including 34 deaths were reported. The outbreak was declared over on 26 November 2012 by the MOH. In Uganda, the last outbreak reported was an outbreak of Sudan ebolavirus in 2022. The last BVD outbreak was recorded in the country in 2007.

This outbreak is occurring in a complex epidemiological and humanitarian context. A critical four-week detection gap between the onset of symptoms of the presumed index case (25 April 2026) and the laboratory confirmation of the outbreak (14 May 2025) suggests a low clinical index of suspicion among healthcare providers. This is compounded by the presence of co-circulating arboviruses and influenza-like illnesses, masking the initial index of suspicion for Ebola disease and exacerbating community transmission. Furthermore, the infection and death of four healthcare workers within a four-day span at Mongbwalu General Referral Hospital underscores critical breaches in IPC protocols. A large number of community deaths has been reported potentially associated with unsafe burial practices.

Ongoing conflict in Ituri province restricts the movement of surveillance teams, limits the deployment of Rapid Response Teams, and hinders the secure transport of laboratory samples. Contact tracing is challenging due to difficult access and highly mobile populations, increasing the risk of high-risk contacts being lost to follow up or never identified

Ituri’s role as a commercial and migratory hub increases the risk of regional exportation. The proximity to Uganda and South Sudan increases the risk of cross-border transmission if PoE screening and cross border coordination and information sharing are not immediately reinforced. On 15 May 2026, the Ministry of Health of Uganda reported an imported case of BVD.

Humanitarian needs in the area are dire. Ituri has 273 403 displaced people, with a total of 1.9 million people in need according to the Humanitarian Response Plan 2026 for DRC. From January to March 2026, 32 600 newly displaced and 30 200 returnees were recorded. The province recorded 5800 protection incidents and 11 incidents against humanitarian actors.

Unlike Ebola virus disease, there is no licensed vaccine or specific therapeutics against BDBV. Research and development activities are activated to coordinate efforts to advance potential candidate medical countermeasures. Response and outbreak control relies entirely on a range of interventions and public health measures that will need to be thoroughly implemented, including supportive care, early detection, adequate IPC, rigorous contact tracing, safe burials, and community engagement.

WHO assessed the risk of the outbreak of BVD to be very high at the national level in DRC, high at the regional level, and low at the global level.

WHO advice

On 19 May 2026, the Director-General of WHO convened the first meeting of the IHR Emergency Committee who issued the following temporary recommendations on 22 May 2026 to State Parties:

For countries with documented detection of Bundibugyo virus (the Democratic Republic of the Congo and Uganda)

Coordination and high-level engagement

- Declare the Bundibugyo virus disease (BVD) epidemic a health emergency, at national or sub-national level, in accordance with domestic laws, and as appropriate.
- Activate national disaster or health emergency management mechanisms and activate or establish an emergency operation centre, under the authority of the Head of State or relevant government authority, to coordinate response activities across Government sectors, administrative levels, and partners to ensure efficient and effective implementation and monitoring of comprehensive BVD control measures. These measures must include enhanced surveillance, including case identification; contact tracing; infection prevention and control (IPC), risk communication and community engagement; laboratory diagnostic testing, case management, and safe and dignified burials. Coordination and response mechanisms should be established at national level, as well as at subnational level in areas where BDBV has been detected and at-risk areas.
- Establish and maintain up to date a register of signals consistent with BVD ("alerts"), including status of their investigation.
- Establish and maintain up to date a line list of suspected cases – including identified through syndromic surveillance, probable cases, and confirmed BVD cases.
- Establish and maintain up to date the list of contacts of all confirmed and probable BVD cases; monitor each contact for 21 days after the date of last known exposure; Both the evolution of the epidemic and resources available may require the risk-based prioritization of contacts requiring identification and monitoring.
- Negotiate, as applicable, and establish security corridors, including cross-border, to allow responders to safely reach affected communities, as well as to allow communities to seek appropriate health care.
- Notify WHO, through the relevant WHO IHR Contact Point in the WHO Regional Office, the detection of suspected, probable and confirmed BVD cases daily, as per WHO case definitions available here.

Risk communication and community engagement

- Implement large-scale trust building and community engagement interventions - using all trusted available communication channels, and working closely with local religious and traditional leaders, and traditional healers -, so that communities are fully aware of the risk and benefits of control measures, and pro-actively contribute and support the early detection and early isolation of cases; the identification and monitoring of contacts; and safe and dignified burial practices

- Strengthen community awareness, engagement and participation, to establish and strengthen trust, including by identifying and addressing cultural norms and beliefs that may serve as barriers to their full participation in the response; and by integrating interventions and community feedback, within the wider response, to address the needs of the population, particularly in contexts of the protracted humanitarian crisis in the Eastern provinces of the Democratic Republic of the Congo.
- Train community leaders on the rationale underpinning public health measures, including the isolation of cases, monitoring of contacts, and safe burials in a dignified, non-stigmatizing, and non-punitive manner.
- Activate local networks, including community health workers, Red Cross volunteers, and other trusted community actors to promote protective behaviours; facilitate early detection and referral of suspected BVD cases; support contact tracing activities; and collect and relay community feedback to enhance the acceptance of public health measures.
- Enable adherence to movement restrictions, associated with the application of control measures, by providing food, water, communication, financial and psychosocial support.

Surveillance and laboratory

- Strengthen surveillance and laboratory capacity, decentralized across first sub-national administrative levels (e.g., provinces) with documented BDBV detection, as well as in their neighbouring first sub-national administrative levels, through:
- Dedicated surveillance and response teams within each health zone and and in neighbouring health zones determined to be at high-risk for the introduction of BVD;
- Active case finding and enhanced community surveillance for clusters of unexplained illness or deaths;
- The investigation of "alerts" within 24 hours from detection;
- The scale-up and strengthen RT-PCR laboratory capacities for timely testing for BDBV, including the establishment of protocols for safe sample collection, sample referral pathways, biosafety training for laboratory workers;
- The decentralization of the laboratory capacities should be considered to allow for quick turn-around time and support patient care, as well as any clinical trials that may take place. Field laboratories should be set-up in accordance with biosecurity and biosafety standards. A near point of care assay might be considered provided that its performance is validated against current RT-PCR standards.
- NB: The GeneXpert platform cannot detect Bundibugyo virus (BDBV).
- Identify and monitor, for 21 days after the date of last known exposure, the health of contacts of suspected probable, and confirmed BVD cases. The health status of contacts being monitored should be assessed and recorded daily. Any contact developing symptoms compatible with BVD should be assessed, isolated, tested and cared for.
- Establish a mechanism to monitor the evolution of indicators related to the performance of contact tracing activities.

Infection prevention and control in health facilities and in the context of care

- Strengthen measures to prevent nosocomial infections, including systematic mapping of health facilities, the establishment and dissemination of protocols for triage, targeted IPC interventions and sustained monitoring and supervision.

- Provide continuous IPC training to health care workers, including the proper use of personal protective equipment (PPE).
- Provide health facilities with sufficient supplies of appropriate PPE equipment to ensure the safety and protection of their staff, resources for timely payment of their salaries and, as appropriate, hazard pay.
- Establish channels for health workers to report and be assessed following exposures, and have access to psychosocial support and, when possible post-exposure prophylaxis under compassionate use or clinical trial. All health worker occupational exposure must be investigated to allow for immediate corrective actions.
- Consider building community IPC capacity by training community leaders, and emphasizing that hand hygiene not only contributes to bringing the BVD epidemic under control, but also reduces the risk of transmission of other communicable diseases present in the same areas. Hand hygiene shall be facilitated at critical spots, such as schools, churches, bars, markets, local gatherings sites, points of entry, etc.

Patient referral pathway and access to safe and optimized intensive care

- Establish dedicated BVD isolation and treatment centers or units for suspected, probable, and confirmed cases, located within, or close to, areas with documented BDBV detection, with sufficient staff who are specifically trained and equipped to implement optimized intensive supportive care.
- Establish protocols for transferring suspected BVD patients safely to dedicated health care facilities for their isolation, assessment and treatment in a humane and patient-centred approach. This includes trained ambulance teams, mechanisms to notify the receiving health care facility, the application of appropriate IPC precautions during transfer, and decontamination protocols for vehicles and equipment.
- Establish protocols for the handling and disposal of medical waste, in accordance with biosafety principles.
- Establish survivor follow-up programmes, including clinical care, counselling, semen testing and sexual health advice and condoms where appropriate, along with psychosocial support and stigma-reduction programmes.
- Maintain the package of essential health services, including providing IPC equipment for them to operate safely. This includes, at minimum, malaria diagnosis and treatment, and maternal and child health services.

Safe and dignified burials

- Establish protocols ensuring funerals and burials are conducted by well-trained personnel, with provision made for the presence of the family and cultural practices, and in accordance with relevant national laws and regulations.

Operations, supplies and logistics

- Establish logistics support to maintain a robust supply pipeline for PPE, diagnostics, therapeutics, and other medical commodities, IPC materials, including for safe burial.

Border health, international travel and mass-gathering events

- Enhance, through arrangements between countries sharing borders, surveillance at ground crossings and border areas.
- Implement measures, in accordance with national laws and regulations, to prevent suspected, probable, and confirmed BVD cases, as well as their contacts from undertaking international travel, unless the travel is part of an appropriate medical evacuation.
- Prevent the cross-border movement of the human remains of deceased suspected, probable or confirmed BVD cases, unless authorized through bilateral arrangements.
- Implement exit screening at all points of entry - airports, ports and ground crossings -, consisting of, at a minimum, a questionnaire encompassing history of potential exposure to BVD, a temperature measurement and, in case of fever, an in depth assessment of the risk of BVD, by personnel trained and equipped with PPE. Any traveller determined to present with an illness consistent with BVD should not be allowed to travel unless the travel is part of an appropriate medical evacuation.
- Report to WHO, through the relevant WHO IHR Contact Point in the WHO Regional Office, the implementation of any international traffic related measure adopted.
- Consider postponing mass gatherings until BVD transmission is interrupted.

Research and development of medical countermeasures

- Engage, when feasible, with research partners and international institutions to:
- Define a robust laboratory strategy, urgently implement head-to-head comparison studies of PCR diagnostics to validate or invalidate the PCR platform (Radione ®) currently used in the field.
- Implement ethically approved, scientifically robust clinical trials to advance the development and use of candidate therapeutics for treatment and post-exposure prophylaxis and for vaccines.
- Establish, with a view to support research, expedited and efficient national regulatory and ethics reviews, community engagement, pharmacovigilance (where applicable), data sharing and equitable access arrangements.

For countries with land borders adjoining countries with documented Bundibugyo virus disease

- Establish a national coordination mechanism articulated with subnational levels.
- Enhance rapidly the status of readiness to respond to BVD cases, including establishing active surveillance across health facilities, with zero reporting; enhancing community-based surveillance for clusters of unexplained deaths; establishing access to laboratories qualified to test for BVD; raising the awareness of health workers regarding BVD; training health workers on IPC precautions; establishing rapid response teams for the investigation and management of BVD patients and their contacts; establishing a mechanism for the identification and monitoring of contacts.
- Establish the capacity at national reference laboratory(ies) to timely and safely perform testing for BDBV along with relevant differential testing. Considerations may be given to shipment to an international reference laboratory for inter-laboratory comparison as part of external quality assurance implementation.
- Conduct international contact tracing operations as necessary, including obtaining information from airlines and other conveyances operations; identifying contacts associated with conveyances on an

international voyage, and communicate with States Parties known as final destination of those contacts.

- Intensify risk communication and community engagement activities, in communities residing in border areas and at points of entry, including airports and ports with direct connection with States Parties with documented BDBV detection, and provide the general public with accurate and up to date information regarding the BVD epidemic and measures to reduce the risk of exposure.
- Exercise arrangements in place to respond to BVD through simulation exercises relating to management of BVD "alerts", including cross-border; sample referral; activation of rapid response teams and mechanisms.
- Establish, with a view to support research, expedited and efficient national regulatory and ethics reviews, community engagement, pharmacovigilance (where applicable), data sharing and equitable access arrangements.
- Border health and international travel
    - Provide travelers with accurate and up to date information regarding the BVD epidemic and measures to reduce the risk of exposure, including discouraging travel to areas with documented BDBV detection.
    - Enhance, through arrangements between countries sharing borders, surveillance at ground crossings. This includes establishing coordination mechanisms for the detection and assessment of travelers with unexplained febrile illness; and the timely sharing of information regarding contacts who have, or may have, crossed the border, thus enabling continuity of follow-up.
    - Pre-position PPE, other IPC materials, sample collection kits, case investigation forms, and safe burial supplies in border areas adjacent to those with documented BDBV detection.
    - Activate health contingency plans at airport and ports, involving conveyance operators, to detect, assess, and manage travellers from States Parties with documented BDBV detection, presenting with symptoms compatible with BVD, and the identification of their contacts, according to established protocols. This entails the availability of trained personnel, referral mechanisms, application of IPC measures.
    - Coordinate with conveyance operators to facilitate timely communication, prior to arrival and to relevant authorities, of any suspected BVD cases on board conveyances, and to identify contacts associated with conveyances on an international voyage. The identification of such contacts entails, where applicable, the communication of personal details to the States Parties known as final destination of those contacts.
    - At the time these temporary recommendations are issued, neither the suspension of flights or waterways routes with States Parties with documented BDBV detection, nor denial of entry to travellers and conveyances arriving from those States Parties, are recommended.
    - Report to WHO, through the relevant WHO IHR Contact Point, the implementation of any international traffic related measure adopted.
- Treat as a health emergency, including through a formal declaration according to domestic laws, the detection of a suspected or confirmed BVD case, of a contact thereof, or of a cluster of unexplained deaths. This include investigating any of those events within 24 hours and, by instituting case isolation and management; establishing a definitive diagnosis; and undertaking the identification and monitoring of contacts.
- Notify to WHO immediately, through the relevant WHO IHR Contact Point in the WHO Regional Offices, any suspected, probable or confirmed BVD case, as per WHO case definitions available here.

In the presence of a BVD case, temporary recommendations for State Parties States Parties with documented BDBV detection apply.

For all other countries

- **Make arrangements to detect, assess, report and manage travelers with unexplained febrile illness arriving from areas with documented BDBV detection. These include, but are not limited to, disseminating the definition of BVD cases to public and private health care facilities, including travel clinics, and general practitioners; identifying laboratories to conduct testing for BDBV; identifying isolation facilities allowing for safe assessment and clinical care.**
- **Provide non-governmental organizations and other entities deploying personnel internationally to respond to the BVD epidemic with information on risk, measures to minimize the risk of exposure, and advice for managing a potential exposure.**
- **Prepare to facilitate the evacuation and repatriation of nationals (e.g., health workers) who have been exposed to BVD cases.**
- **Provide the general public with accurate and up to date information regarding the BVD epidemic and measures to reduce the risk of exposure, including discouraging travel to areas with documented BDBV detection.**
- **Border health and international travel**
  - **Provide accurate and up to date information regarding the BVD epidemic to travel clinics, other health facilities and professionals, and discourage travel to areas with documented BDBV detection.**
  - **Provide incoming travelers, at points of entry, with information about measures to take should they develop symptoms compatible with BVD within 21 days after arrival.**
  - **Coordinate with the transport sector, including conveyance and points of entry operators, for the timely management of suspected BVD cases, including communication prior to arrival if the individual is on board; as well as for the identification of their contacts on board conveyance. The identification of such contacts entails, where applicable, the communication of personal details to the States Parties known as final destination of those contacts.**
  - **At the time these temporary recommendations are issued, neither the suspension of flights from States Parties with documented BDBV detection, nor denial of entry to travellers and conveyances arriving from those States Parties, are recommended.**
- **Report to WHO, through the relevant WHO IHR Contact Point, the implementation of any international traffic related measure adopted.**
  - **Notify to WHO immediately, through the relevant WHO IHR Contact Point in the WHO Regional Offices, any suspected, probable or confirmed BVD case, as per WHO case definitions available here.**

In the presence of a BVD case, temporary recommendations for States Parties with documented BDBV detection apply.

Regular Information products on the outbreak of BVD in DRC

- Daily update: Epidemiological update on BVD outbreak in DRC and Uganda
- Published every Tuesday: Weekly External Situation Report on Ebola Bundibugyo Virus Disease Outbreak, Democratic Republic of the Congo | Uganda
- Published every Thursday:Disease Outbreak News | All Hazards Public Health Events, Ebola disease caused by Bundibugyo virus, Democratic Republic of the Congo

Further information

- Epidemic of Ebola Disease caused by Bundibugyo virus in the Democratic Republic of the Congo and Uganda determined a public health emergency of international concern
- The Ministry of Public Health, Hygiene and Social Welfare, DRC, officially declares the 17th Ebola Disease outbreak
- WHO Democratic Republic of Congo confirms new Ebola outbreak
- Weekly External Situation Report. EBOLA BUNDIBUGYO VIRUS DISEASE OUTBREAK Democratic Republic of the Congo | Uganda.
- Ebola disease fact sheet: http://www.who.int/en/news-room/fact-sheets/detail/ebola-virus-disease
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